



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض رباعياً : صباح سلومه عجاب لطفي  
المحافظة : القاهرة  
الحى / المدينة : حلوان  
رقم البطاقة : 28306062300407 رقم التليفون : 01126015120  
العمر (تاريخ الميلاد) : 06-06-1983 المهنة : بدون عمل  
العنوان : 34 ش حسين زكى  
تاريخ تسجيل الإستمارة : 02-10-2026

### الإقرار

أقر أنا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (.....)  
المقرر بما فيه  
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)  
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)  
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام  
التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني  
الاسم : د/ندى حسن  
الاسم : د/نرمين رمضان  
الوظيفة : .....  
الوظيفة : .....  
الوظيفة : .....  
التوقيع : .....  
التوقيع : .....  
التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.  
تعتمد صحة الأبحاث والمرفقة  
رئيس اللجنة ومدير  
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

..... د/ ..... د/ ..... د/ ..... د/

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : .....  
جهة العلاج : .....  
ملاحظات : .....  
قسم : .....  
نفقات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

Name: صباح سلومة عجاب لظف  
National ID: 28306062300407



## تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المريضة تعاني من ورم غير حميد بالبدن و تحتاج الي Further investigations and imaging

الوقت :

---:--

التاريخ :

05-07-2026

الطبيب المعالج :

د/شيماء لاثنين



02/28/2026

**MDT FORM**

|                 |                        |                 |                    |
|-----------------|------------------------|-----------------|--------------------|
| Name:           | Sabah Sioma Egab Lotfe | MRN:            | 4083               |
| Age:            | 43 Y                   | ID:             | 28306062300407     |
| Menopausal Sta: | Premenopausi           | Phone:          | 01091512476        |
| NRS SCORE:      |                        | Marital Status: |                    |
| BMI:            |                        | Family History: | No                 |
| CO Morbidity:   |                        | Reason Of MDT:  | Systemic treatment |

**Clinical Data:**

BX: IDC  
ER: 7/8 PR: 2/8 HER2: 1+ KI67:

R

L

**Sonomammography:**

**Mets Workup:**

**History:**

A 44-year-old female diagnosed in 2019 with right breast cancer. She received NACT in the form of AC followed by Taxol, then underwent right BCS with SLNB followed by PORT. She was subsequently maintained on adjuvant endocrine therapy with aromatase inhibitor (AI) plus Zoladex for four years. In 2024, she developed metastatic disease involving the bone and lungs. She was treated with palbociclib in combination with Fulvestrant for six months; however, bone scan showed disease progression. Accordingly, treatment was shifted to (Aromasin) plus (Afinitor) with (Zoladex), Received for 7 months

Bone Scan 29/12/2025 : SD of multiple bony lesions

CT Brain: Rt frontal and Rt temporal SOL with extracranial and intraorbital extension. Did Brain MRI pending results.

O/E: Multiple new cervical LN's. For MDT to shift ttt.

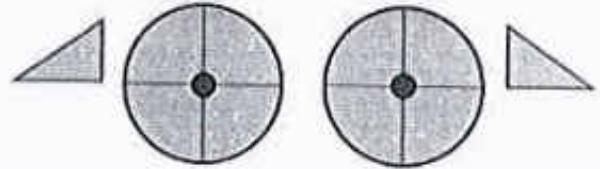
**Recommendations:**

Oral navelbine + Zometa

**MDT Outcome & Plan**

FOR REBIOPSY

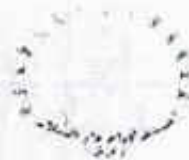
MEANWHILE TO START TAXOTER SINGLE AGENT



TNM

**SIGNATURE**

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|------------------|---------|-----------|-----------|-----------|-------------|
| د. هاني حريت     |         |           |           |           |             |



|                                         |                   |
|-----------------------------------------|-------------------|
| Patient Name: Sabah Sallouma Agab Lotfy | Date : 22/02/2025 |
| Exam : C.T CHEST WITH CONTRAST          | Age : 41 Year(s)  |
| Medical ID: 3668/2024                   | Pat. ID: 97876    |

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

**\*\*known case of right conservative breast surgery on hormonal treatment:**

**1. Chest:**

- Right breast LIQ surgical clip with no sizable masses.
- Few subcentemetric right axillary level I lymph nodes.
- Few prominent bilateral internal mammary lymph nodes, largest on the right side measures about 8mm.
- Enlarged left axillary lymph nodes largest 2.9x1.5 cm.
- Prominent right cardiophrenic lymph node, measures about 9 x6 mm.
- Multiple bilateral scattered pulmonary nodules, the largest pleural based lingular inferior segment measuring 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

**Side finding:** Enlarged left thyroid lobe showing partially cystic nodule, for ultrasonographic assessment.

**2. Abdomen and pelvis:**

- The liver is average in size showing few bilobar hepatic focal lesions largest is noted at segment V/VI measuring about 1.3 x 1.3 cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**3. The scanned skeleton:**

- Diffuse sclerotic osseous patchy areas and foci involving sternum, multilevel dorso-lumbar vertebrae, left sacrum, right acetabulum anterior column as well as right greater trochanter.
- Right internal mammary retrosternal extra-osseous soft tissue mass lesion, measuring about 4.5 x 3 cm seen entangling the right internal mammary vessels, associated with significant erosive changes of the related sternal border.



|                                         |                   |
|-----------------------------------------|-------------------|
| Patient Name: Sabah Sallouma Agab Lotfy | Date : 22/02/2025 |
| Exam : C.T CHEST WITH CONTRAST          | Age : 41 Year(s)  |
| Medical ID: 3668/2024                   | Pat. ID: 97876    |

- L1 schmorl's node /central depression.

**OPINION:**

**\*\*known case of right conservative breast surgery on hormonal treatment.**

- ***Multiple bilateral scattered pulmonary nodules, ?? metastatic nature.***
- ***Few bilobar hepatic focal lesions, Likely metastatic, for triphasic CT assessment.***
- ***Diffuse sclerotic osseous patchy areas involving sternum, multilevel dorso-lumbar vertebral and pelvic bones with right retrosternal extra-osseous soft tissue mass lesion, mostly of metastatic nature.***

***Dr.Menna Salem, MSc.***

***Dr. Walid Mohamed, M.D.***



Presidential Initiative for Women's Health  
Pathology Lab - Cairo University Hospitals  
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي  
مستشفيات جامعة القاهرة



**PATHOLOGY REPORT**

Name: صباح سلومة عجاب لطفي  
Patient's I.D.: 28306062300407  
Age: 41 Y.

Pathology Number: 252 /2024  
Specimen Received Data: 30 / 07 /2024  
Date Report Issued: 12/ 08 /2024

Clinical data: H/O right cancer breast with CBS.  
Nature of the specimen: Left axillary lymph node, U.S. Guided Tru Cut Biopsy.

**GROSS EXAMINATION:** Tissue core biopsy, totally submitted.

**MICROSCOPIC EXAMINATION:** Serial sections examined revealed cores of nodal tissue showed effacement of architecture and infiltration by malignant tumor formed of trabeculae and groups as well as single and signet ring forms of markedly anaplastic ductal epithelial cells.

**IMMUNOHISTOCHEMISTRY PROCEDURE:** Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system, DAB was used as a chromogen and hematoxylin as a counter stain. Tissue sections from paraffin block were used for quantitative detection of HER2 amplification using Ventana Benchmark Ultra System autostainer & VENTANA INFORM HER2 Dual DNA Probe Cocktail Assay.

**IMMUNOHISTOCHEMISTRY RESULTS:**

Estrogen receptors: moderate positive nuclear staining in most of tumor cells.  
Progesterone receptors: weak positive nuclear staining in <1% of most of tumor cells.  
Her-2/neu: Weak to moderate incomplete membrane staining in >10% of tumor cells (score 1+).  
Ki67: Positive nuclear stain in average of ±80% of tumor cells.

**DIAGNOSIS:** Left Axillary Lymph Node, U.S. Guided Tru Cut Biopsy;  
Metastatic Carcinoma, Compatible With Mammary Duct Origin.

- **Positive** For Estrogen Receptors, Score 7/8.
- **Negative** For Progesterone Receptors, Score 2/8.
- **Low Positive** For Her-2/neu over-expression (Score 1+).
- **80%** Ki67 Proliferative Index.

PROF. DR. AHMED ABDEL AZIZ  
DR. PASSANT ESSAM SHIBEL

اتاهى به العبد  
1.9.10.2024  
ن. الرضا

Name : Sabah Salouma Ajeeb Lotfy  
Branch : Dokki

Date : 05-09-2025 10:53  
Medical No. : 195034

### Tc<sup>99m</sup> MDP SKELETAL SCINTIGRAPHY

#### **History:**

- 44-year-old lady, with history of metastatic breast cancer, referred for follow up.

#### **Technique:**

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

#### **Findings:**

- Considering the previous bone scan dated 14/10/2024, the current study revealed:
  - Newly developed active lesions at the skull, proximal shaft of right humerus, multiple dorso-lumbar vertebrae (with progressed old ones), pelvic bones and trochanteric region of right femur.
- Degenerative changes are seen at the spine and big joints.

#### **Impression:**

- Metastatic osseous lesions with progressive course evidenced by multiple newly-developed deposits.

Best regards,  
Dr. Alaa Nagi, MD  
Nuclear Medicine



|                                         |                   |
|-----------------------------------------|-------------------|
| Patient Name: Sabah Sallouma Agab Lotfy | Date : 09/07/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis)  | Age : 42 Year(s)  |
| Medical ID: 3668/2024                   | Pat. ID: 97876    |

**EXAMINATION: MULTI-SLICE -CONTRAST C.T. CHEST,  
ABDOMEN AND PELVIS**

*Known case of BC with left mastectomy :*

- \*Clear operative bed , no masses .
- \*No evidence of pulmonary cavitating lesions.
- \* Few millimetric bilateral lung nodules .
- \*Anterior subpleural areas of nodular infiltration with fine reticulations are seen at left upper lob .likely radiation pneumonitis.
- \*few mediastinal enlarged lymph nodes the largest seen at subcarinal group measures 6mm .
- \*Clear costophrenic recesses .
- \*Scanned vertebrae and ribs are free.

**Abdomen and pelvis:**

- The liver is average in size with no sizable focal lesions based on non-contrast study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and right supra-renal.
- Normal appearance of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Bony Skeleton: L1 small sclerotic lesion likely benign.

**Opinion:**

- Picture of radiation pneumonitis for clinical correlation.

DR/ Reham Khedr ...MD.

9/8/26, 4:50 PM

code:02 - Name\_A MDT - Name\_E MDT

د. مصطفى  
جوانى  
أ. - ك



مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

الجمهورية العربية السورية  
سجل  
الطب

09/08/2026

## MDT FORM

|                |                        |                 |                |
|----------------|------------------------|-----------------|----------------|
| Name:          | Sahar Siamra Egab Lode | MRN:            | 4083           |
| Age:           | 43 Y                   | ID:             | 26306062300407 |
| Menopausal Sta |                        | Phone:          | 01126025820    |
| NRS SCORE:     |                        | Marital Status: |                |
| BMI:           |                        | Family History: | -ve            |
| CO Morbidity:  | free                   | Reason OF MDT:  | dp             |

Clinical Data:

BX:

ER: 7/8

PR: 2/8

HER2: +1

KI67:

R

L

Sonomammography:

Mets Workup:

History:

A 44-year-old female diagnosed in 2019 with right breast cancer. She received NACT in the form of AC followed by Taxol, then underwent right BCS with SLNB followed by PORT. She was subsequently maintained on adjuvant endocrine therapy with aromatase inhibitor (AI) plus Zoladex for four years. In 2024, she developed metastatic disease involving the bone and lungs. She was treated with palbocicli in combination with fulvestrant for six months, however, bone scan showed disease progression. Accordingly, treatment was shifted to afnitor/aromasin till 3/2025. complained of headache and blurring of vision >> mri brain skull bony mets extraxial sol nec wcl 3/2025  
new ct >> ad with HFLs  
new biopsy from HFL 7/2026 >> met carcinoma of breast origin grade 3. er 6/8 pr0/8 her2-ve  
then shifted to taxotere received bc with bone scan showed PD  
ECOG >> 3  
PT complain of severe pain at RT hemipelvis region + lumbar region  
**Recommendations:**

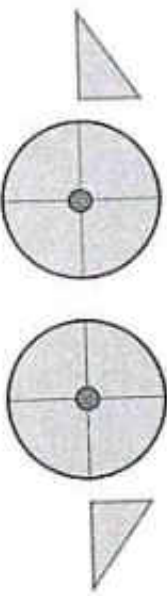
**MDT Outcome & Plan**

For palliative Radiotherapy  
If PS improves --> For oral Navelbine + Zometa

SIGNATURE

|                  |         |           |           |           |             |
|------------------|---------|-----------|-----------|-----------|-------------|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
|                  |         |           |           |           |             |

T N M





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Patient's Name: Sabah Saloma Ajeeb Lotfi  
Date: 2026-07-13  
Referred by: Prof.Dr. no doctor  
Hospital: Goustave Roussey International Egypt

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -**

**Clinical History:**

- History of breast cancer
- compared to last study dated 26/4/2026 current study revealed:

**CHEST**

- Clear both breasts and right axilla.
- Still seen left axillary enlarged suspicious LNs for US correlation
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules of average size 2-3mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

**ABDOMEN AND PELVIS: -**

- The liver is of average size with homogenous CT texture with decrease in size of the previously mentioned bilobar multiple hepatic focal lesions with peripheral enhancement largest is seen at segment III measuring 20x19mm compared to 2.9x3cm at previous study
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

**THE EXAMINED SKELETON**

- Still seen multiple scattered lytic and sclerotic mainly sclerotic bony lesions are seen along the axial and appendicular skeleton denoting metastasis.

**Opinion:**

- Regressive course of hepatic focal lesions
- Stationary rest of study.

DR/NERMEEN RAMADAN.MD

Name : Sabah Saloma Ajeeb Lotfi

Branch : Dokki

Ref.Physician Prof: Goustav Roussey Egypt

Date : 23-07-2026 04:12

Medical No. : 423082

## WHOLE-BODY BONE SCINTIGRAPHY REPORT

Indication: Follow-up of metastatic breast carcinoma.

Comparison: Prior bone scan dated September 5, 2025.

### PROCEDURE

Planar anterior and posterior whole-body images following IV 99mTc-MDP.

### FINDINGS

Skull: Interval regression of calvarial lesions compared to prior study.

Spine: Significant progression with multiple new intensely active osteoblastic lesions throughout cervical, thoracic, and lumbar spine.

Pelvis: Progressive disease with new foci involving both iliac bones, acetabula, pubic rami, and sacrum.

Upper Extremities: New prominent lesions in proximal humeri bilaterally (right > left).

Lower Extremities: New focal uptake at right femoral trochanter and proximal shafts.

Kidneys & Bladder: Physiological excretion.

|                 |                       |                 |                          |
|-----------------|-----------------------|-----------------|--------------------------|
| Patient Name    | صباح سلوة عجل         | Case No.        | 276242                   |
| Sex/Age         | Female 43 Y           | Patient No.     | 20691                    |
| Registered Date | 13/6/2026 09:48:33 AM | Referred By Dr. | Interventional Radiology |
| Validation Date | 05/7/2026 01:33:58 PM | Branch          | Gustave Roussy           |
| Reporting Date  | 13/7/2026 10:59:27 AM | Pathology No.   | 1859/26                  |

**Clinical Data:** Hepatic focal lesion in a patient with previous history of breast cancer

**Nature of Specimen:** Tru-cut biopsy, collected on 13/06/2026, at 10:20 am. Cold ischemia time is 20 seconds.

**Technique:**

- Technique:**  
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for:  
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & GATA3 clone (L50-823).  
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & GATA3 clone (L50-823).

**Microscopic examination:**

**Microscopic examination:**  
-Histopathological examination reveals cores of hepatic tissue showing metastatic breast carcinoma, grade 3.

### Results:

- \* IPT (All controls showed appropriate reactivity):
- GATA3: Strong diffuse positive staining in tumor cells.
- ER: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).

### Conclusion:

**Hepatic focal lesion, Tru-cut biopsy:**

- Consistent with metastatic deposits from breast carcinoma, grade 3.

## IPT:

- GATA3: Positive.
- ER: Positive (score 6/8).
- PR: Negative (score 0/8).
- Her2: Ultra-Low (score 0+).

<sup>a</sup>Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2,



intermediate; and 3, strong). Total score from 0 to 8.

**\*\* Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+, based on circumferential membrane staining that is incomplete and/or weak/moderate and within  $\geq 10\%$  of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within  $<10\%$  of the invasive tumor cells. IHC 3+, based on circumferential membrane staining that is complete, intense  $>10\%$  of tumor cells.

Signature \_\_\_\_\_

*Dr. Walaa Ibrahim Ghoneim, MD.*



دوسلاف  
رؤسبي  
أشرف  
أشرف

Patient's Name:

Sabah Saloma Ajeeb Lotfi

Date:

2026-07-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



#### MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Clear operative bed in the breast with no evidences of either residual and /or recurrent neoplastic lesions seen.
- Normal Both axillae.
- Hyperinflated both lungs.
- Few enhancing tiny subpleural pulmonary nodules are seen in the medial and in the lateral segment of left lower pulmonary lobes with the largest one measures about 5.4mm in diameter.
- No other sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- Foci of osseous sclerotic lesions and to lesser extent lytic lesions are seen scattering in the skeleton.
- Order line size of the liver,measuring about 15.8 cm, showing a homogenous parenchymal attenuation and enhancement pattern. A tiny hypovascular subcapsular focal lesion is seen in segment VIII of the right lobe and measures about 16x16mm in dimensions.
- No other sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Border line sized spleen,measuring about 13 cm. No focal lesions seen inside.
- Relatively thickened left adrenal gland.
- Normal CT appearance of the pancreas, functioning both kidneys(apart from a tiny cortical cyst in the lower pole of left kidney and measures about 16x13mm in dimensions), right adrenal gland,aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascies.



Patient's Name:

Sabah Saloma Ajeeb Lotti

Date:

2026-07-13

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- No intra-vesical stones, solid masses, wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

#### IMPRESSIONS:

- Clear operative bed in the breast with no evidences of either residual and /or recurrent neoplastic lesions seen and normal Both axillae.
- Hyperinflated both lungs with few tiny enhancing metastatic subpleural pulmonary nodules bilaterally, foci of osseous sclerotic lesions and to lesser extent lytic lesions scattering in the skeleton, denoting osseous deposits, a tiny hypovascular subcapsular focal lesion in segment VIII of the right hepatic lobe, likely to be a metastasis and relatively thickened left adrenal gland, denoting a metastasis or a hyperplasia.

Prof/Hanan El-Ahwal.

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Patient's Name:

SABAH SALOMA AGAB LOTFY

Date:

2026-04-26

Referred by:

Prof.Dr. CT CAP IV

Hospital:

Goustave Roussey International Egypt

## **POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-**

### **Clinical History:**

- **History of breast cancer underwent lumpectomy, compared to last study dated 9/7/2025 current study revealed:**

### **Findings:**

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs are seen largest is measuring 2.2x2.5cm.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules of average size 2-3mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

### **OPINION:**

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules.

## **MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-**

### **Findings:**

Patient's Name: SABAH SALOMA AGAB LOTFY  
Date: 2026-04-26  
Referred by: Prof.Dr. CT CAP IV  
Hospital: Goustave Roussey International Egypt

- The liver is of average size with homogenous CT texture with bilobar multiple hepatic focal lesions with peripheral enhancement largest is seen at segment III measuring 2.9x3cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Multiple scattered lytic and sclerotic mainly sclerotic bony lesions are seen along the axial and appendicular skeleton denoting metastasis.

#### Opinion:

- Newly developed hepatic and bony metastasis as detailed.

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban



Patient's Name:

SABAH SALOMA AGAB LOTFY

Date:

2026-04-26

Referred by:

Prof.Dr. CT CAP IV  
Gonstave Roussey International Egypt

Hospital:

## **POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-**

### **Clinical History:**

- **History of breast cancer underwent lumpectomy, compared to last study dated 9/7/2025 current study revealed:**

### **Findings:**

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs are seen largest is measuring 2.2x2.5cm.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules of average size 2-3mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

### **OPINION:**

- Clear both breasts and right axilla.
- Left axillary enlarged suspicious LNs.
- Stationary course as regards the size and the number of bilateral few scattered pulmonary nodules.

## **MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-**

### **Findings:**

Patient's Name: SABAH SALOMA AGAB LOTFY  
Date: 2026-04-26  
Referred by: Prof.Dr. CT CAP IV  
Hospital: Goustave Roussey International Egypt

- The liver is of average size with homogenous CT texture with bilobar multiple hepatic focal lesions with peripheral enhancement largest is seen at segment III measuring 2.9x3cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Multiple scattered lytic and sclerotic mainly sclerotic bony lesions are seen along the axial and appendicular skeleton denoting metastasis.

#### Opinion:

- **Newly developed hepatic and bony metastasis as detailed.**

**Dr. Mai Shaaban MD**

*Dr.Mai Shaaban*

*Dr.Mai Shaaban*

2. 12

**GUSTAVE /  
ROUSSY**  
INTERNATIONAL  
ALGERIE

|                                          |                                         |
|------------------------------------------|-----------------------------------------|
| <b>Name :</b> SABAH SALOMA OGAAB         | <b>Date :</b> 14-02-2026 04:54          |
| <b>Branch :</b> Portable                 | <b>Medical No. :</b> GRI-28306062300407 |
| <b>Ref.Physician Prof :</b> MRI BRAIN +C | <b>Ins.Company :</b>                    |

## **FOLLOW UP MRI OF THE BRAIN**

### **TECHNIQUE:**

Axial T1, T2, FLAIR, diffusion & ADC WIs  
Coronal T2WIs  
Sagittal T1WIs  
Post contrast T1 WIs

### **FINDINGS:**

- Few right and left dural based extra-axial space occupying lesions are seen in the right fronto-temporal, right high parietal and left fronto-parietal region with all of them invading the overlying skull bone and scalp with contour bulge impressive of metastatic brain and skull deposits. They are seen eliciting isointense signal in T1, mixed iso and high signal in T2 and FLAIR WIs with post contrast enhancement.



- The largest lesion is seen dural based at the right fronto-temporal region measuring 6.3 x 3 x 4.6 cm at its maximal CC, TV and AP diameters.
- Symmetrical appearance of the ventricular system.
- No intra or extra axial areas of fresh blood signal intensities or its degradation products.
- The brain stem and cerebellum are normal.

### **OPINION:**

- No available previous studies showing few bilateral metastatic deposits in skull bone and extra-axial brain deposits.

*Much Obliged;*  
**Dr.Marwa Elgawish, MSc, FEBR**

MDT FORM

MDT FORM

MDT FORM

|                          |                 |                       |              |
|--------------------------|-----------------|-----------------------|--------------|
| <u>Name</u>              | Dr. [Signature] | <u>MRN</u>            | 283060623004 |
| <u>Age</u>               | 43 yrs          | <u>ID</u>             | 47711/C.C.E  |
| <u>Menopausal status</u> | perimenopausal  | <u>Phone</u>          | 01091512476  |
| <u>NRS SCORE</u>         |                 | <u>Marital status</u> |              |
| <u>BMI</u>               |                 | <u>Family history</u> | -            |
| <u>CO morbidity</u>      | No              | <u>REASON OF MDT</u>  |              |

### Clinical Data

|                                                                                                     |                       |                         |              |
|-----------------------------------------------------------------------------------------------------|-----------------------|-------------------------|--------------|
| <u>BX:</u> <u>IOC</u>                                                                               |                       |                         |              |
| <u>ER:</u> <u>7/8</u>                                                                               | <u>PR:</u> <u>2/8</u> | <u>HER 2:</u> <u>1+</u> | <u>Ki67:</u> |
| <u>Sonomammography:</u>                                                                             |                       |                         |              |
| <u>Mets Workup:</u><br>(See scan 12/9/2023) → progressive<br>Cans with newly developed<br>deposits. |                       |                         |              |

**Recommendations:**

2019 11 Break IDC (EE 710 SR 248 42 1- K678571)

$\text{reduced water} \rightarrow \text{RT-BL} \rightarrow \text{PBT} \rightarrow \text{Tubex / XI}$

2021 while on 2010s/11E → base of W mts → Ax W br  
Fox 62 7/8 PR 218 H-21+ → 12 line Foshan + JTBanc

## MDT OUTCOME &amp; PLAN

(read 6 only but only one base is in 10/2020) → 179/2020 Base is

Consider Shift to Black / Aromatic / Alkyne.

**SIGNATURE**

| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY  | PATHOLOGY  | Nutrition | Psychiatric |
|------------------|---------|------------|------------|-----------|-------------|
| 5 Dec 1968       | 10      | 1 Argemose | 1 Argemose | 166666    |             |

20/12/20

MDT FORM 22 / 15 / 2024

|                   |               |                |             |
|-------------------|---------------|----------------|-------------|
| Name              | صباح سلومة    | MRN            | 61726       |
| Age               | 41            | National ID    |             |
| Menopausal status | premenopausal | Phone          | 01031512476 |
| Marital status    | married       | Wt., Ht.       |             |
| Co morbidities    | nil           | Family history | -ve         |

### Clinical Data

Met. Breast CA  
Bx: mets CA compatible e'mammory duct

ER: +ve PR: -ve HER2: t+ Ki67: 80%  
7/8 2/8

Sonomammography: Left axillary level I & II LNs  
4x1.6cm e' cortical thickness 1.2

Lt Breast: free

Rt Breast: clear axillary

Mets Worked: Bone scan -> osseous lesions in

sternum, L V1, L V4

CT CAP: -> soft tissue lesion at 3rd L4/5 vertebral

5.4x3.3 cm, & axillary LNs, few bilateral small pleural nodules

few pleural nodules

Recommendations and further workup:

fluorouracil in 2019. RT IDC G2 ER 8/8 PR 8/8 Her 2-ve Ki67 20%

NACT ACx4, total 12 wks underwent at BCS & SLNB by PT-1 N1b (2/14)

received PORTH (breast DC LNs) on AI zoladex for 4 years.

then C/O of axillary recurrence.

Bx: IDC, ER 9/8 PR 2/8 HER2 -1

In view of relapse of on city Zoladex-AI

### MDT OUTCOME & PLAN

for - 2nd line Fasldep - Zoladex - Ibrance  
+ Zometa 13m

| SIGNATURE        |           |           |           |
|------------------|-----------|-----------|-----------|
| MEDICAL ONCOLOGY | SURGERY   | RADIOLOGY | PATHOLOGY |
| psychiatry       | Nutrition |           |           |

Name : Sabah Salouma Ajeeb Lotfy  
Branch : Dokki

Date : 05-09-2025 10:53  
Medical No. : 195034

## Tc<sup>99m</sup> MDP SKELETAL SCINTIGRAPHY

### **History:**

- 44-year-old lady, with history of metastatic breast cancer, referred for follow up.

### **Technique:**

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

### **Findings:**

- Considering the previous bone scan dated 14/10/2024, the current study revealed:
  - Newly developed active lesions at the skull, proximal shaft of right humerus, multiple dorso-lumbar vertebrae (with progressed old ones), pelvic bones and trochanteric region of right femur.
- Degenerative changes are seen at the spine and big joints.

### **Impression:**

- Metastatic osseous lesions with progressive course evidenced by multiple newly-developed deposits.

Best regards,  
Dr. Alaa Nagi, MD  
Nuclear Medicine



Presidential Initiative for Women's Health  
Pathology Lab - Cairo University Hospitals  
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي  
مستشفيات جامعة القاهرة



**PATHOLOGY REPORT**

Name: صباح سلومه عجاب لطفي  
Patient's I.D.: 28306062300407  
Age, 41 Y.

Pathology Number: 252 /2024  
Specimen Received Data: 30 / 07 /2024  
Date Report Issued: 13/ 08 /2024

Clinical data: H/O right cancer breast with CBS.

Nature of the specimen: Left axillary lymph node, U.S. Guided Tru Cut Biopsy.

**GROSS EXAMINATION:** Tissue core biopsy, totally submitted.

**MICROSCOPIC EXAMINATION:** Serial sections examined revealed cores of nodal tissue showed effacement of architecture and infiltration by malignant tumor formed of trabeculae and groups as well as single and signet ring forms of markedly anaplastic ductal epithelial cells.

**IMMUNOHISTOCHEMISTRY PROCEDURE:** Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain. Tissue sections from paraffin block were used for quantitative detection of HER2 amplification using Ventana Benchmark Ultra System autostainer & VENTANA INFORM HER2 Dual DNA Probe Cocktail Assay.

**IMMUNOHISTOCHEMISTRY RESULTS:**

Estrogen receptors: moderate positive nuclear staining in most of tumor cells.  
Progesterone receptors: weak positive nuclear staining in <1% of most of tumor cells.  
Her-2/neu: Weak to moderate incomplete membrane staining in >10% of tumor cells (score 1+).

Ki67: Positive nuclear stain in average of  $\pm 80\%$  of tumor cells.

**DIAGNOSIS:** Left Axillary Lymph Node, U.S. Guided Tru Cut Biopsy.

**Metastatic Carcinoma, Compatible With Mammary Duct Origin.**

- **Positive** For Estrogen Receptors, Score 7/8.
- **Negative** For Progesterone Receptors, Score 2/8.
- **Low Positive** For Her-2/neu over-expression (Score 1+).
- **80% Ki67** Proliferative index.

PROF. DR. AHMED ABDEL AZIZ  
DR. PASSANT ESSAM SHIBEL

أستاذة  
أ. شيبان  
أ. شيبان  
أ. شيبان  
أ. شيبان

جمهورية مصر العربية  
بطاقة تعريف الشخصية



صباح  
سلومة عجاب لطفى  
٣٤ ش حسين زكى جزيرة الذهب  
الجيزة - الجيزة

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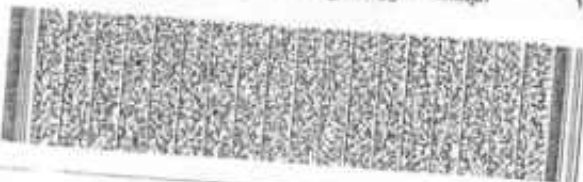
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بدون عمل

النسب مسلمة  
السيد محمد عبدالله عطيه  
الهيئته سارية حتى ٢٠٢٥/١٢/١٦



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مفت ملين ش



**Name: Sabah Salouma Ogab**

**Date: 28/8/2024**

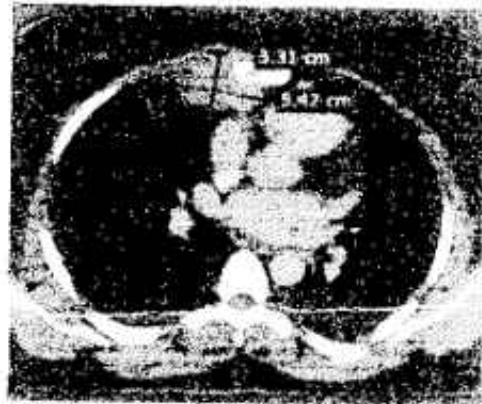
**POST-CONTRAST CT SCAN OF THE CHEST:-**

**\*\* Reported case of breast cancer underwent neoadjuvant chemotherapy with CBS:**

**\*\* Examination is degraded by the limited number of chest cuts hindering proper assessment:**

**Findings:**

- Lobulated soft tissue lesion seen implicating the right parasternal region medial to the 3<sup>rd</sup> and 4<sup>th</sup> costo-chondral junctions, measuring about 5.4x3.3 cm in axial dimensions, seen abutting the sternal body and the right internal mammary vessels.
- Multiple enlarged left axillary lymph nodes, some of them showing globular outline, the largest measuring about 4x1.6 cm in axial dimensions.
- Few bilateral small predominantly subpleural and fissural nodules, the largest measuring about 4 mm in diameter.
- Lingular parenchymal band.
- No consolidation, cavitation or bronchiectatic changes.
- Few prominent prevascular and paratracheal lymph nodes, with subcentimetric right cardiophrenic lymph node.
- No pleural or pericardial collections.
- No gross cardiac abnormality.
- Right breast dense metallic structure, likely small surgical clip.



**Dr/ Aya Hussein, MD**

**Name: Sabah Salouma Ogab**

**Date: 28/8/2024**

**CECT SCAN OF THE ABDOMEN AND PELVIS: -**

**\*\* Known case of cancer breast underwent neoadjuvant chemotherapy with CBS:**

**\*\* Examination is degraded by limited number of abdominal cuts hindering proper parenchymal assessment:**

**Findings:**

- Mildly enlarged liver showing smooth outline with homogenous parenchymal texture. No sizable focal lesions. No biliary radicles dilatation.
- Normal CT appearance of the spleen, kidneys, adrenal glands and pancreas.
- Average filled urinary bladder with no sizable mass lesions or diverticular outpouchings.
- Few prominent bilateral iliac lymph nodes. No sizable pathologically enlarged abdominal lymph nodes.
- No ascites.

**\*\* For correlation with clinical data and previous unavailable films.**

**Dr/ Aya Hussein, MD**